

Patient ID: WheelerPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

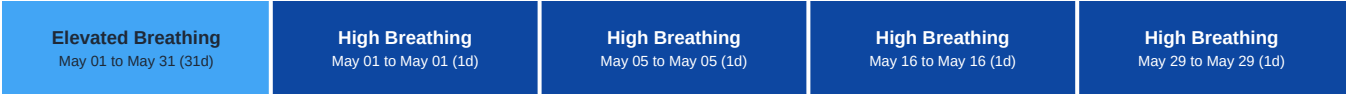
Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Patient clinical status over 32 days from May 01 to Jun 01

HR Breathing



Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: RED

Vital	Avg	Min	Max
Heart Rate	59 bpm	51 bpm	71 bpm
Breathing	22 brpm	12 brpm	51 brpm

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 31	2 AM to 7 AM	5h	1	Elevated Breathing	26	14	51	Check SpO2 and lung sounds; assess for fluid overload or early infection

77 episodic events Detected, spanning 11 days total. Conditions: Elevated Breathing, High Breathing. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
77	278h	Elevated Breathing	2	100%

Major Findings: Sustained elevated breathing (avg 26, peak 51)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 01	7 AM to 11 AM	4h	1	High Breathing	31	17	34	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	9 PM to 10 PM	1h	1	High Breathing	32	12	49	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 16	8 AM to 9 AM	4h	2	High Breathing	31	17	36	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 29	6 AM to 7 AM	1h	1	High Breathing	32	27	36	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 01	3 AM to 7 AM	268h	72	Elevated Breathing	26	8	51	Check SpO2 and lung sounds; assess for fluid overload or early infection

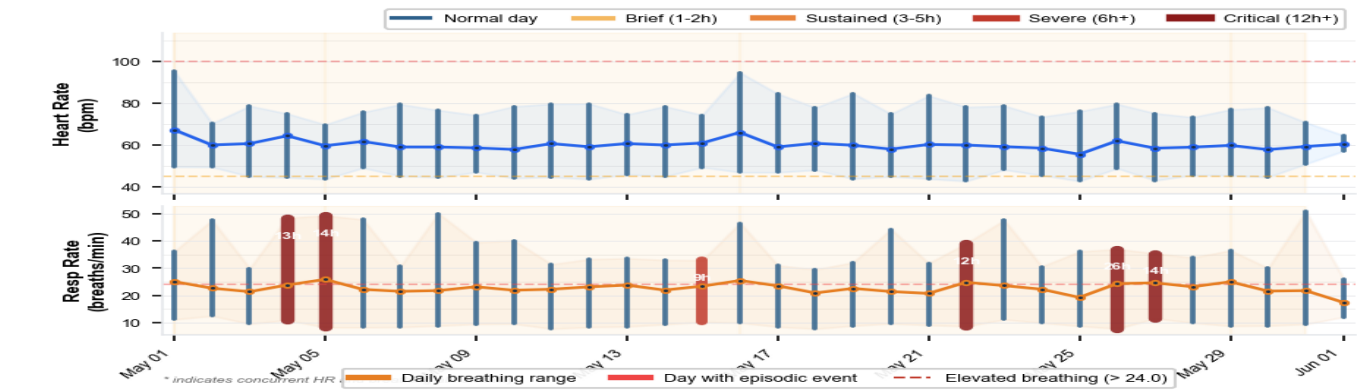
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 51 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 34 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 5 unstable phases with 13 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

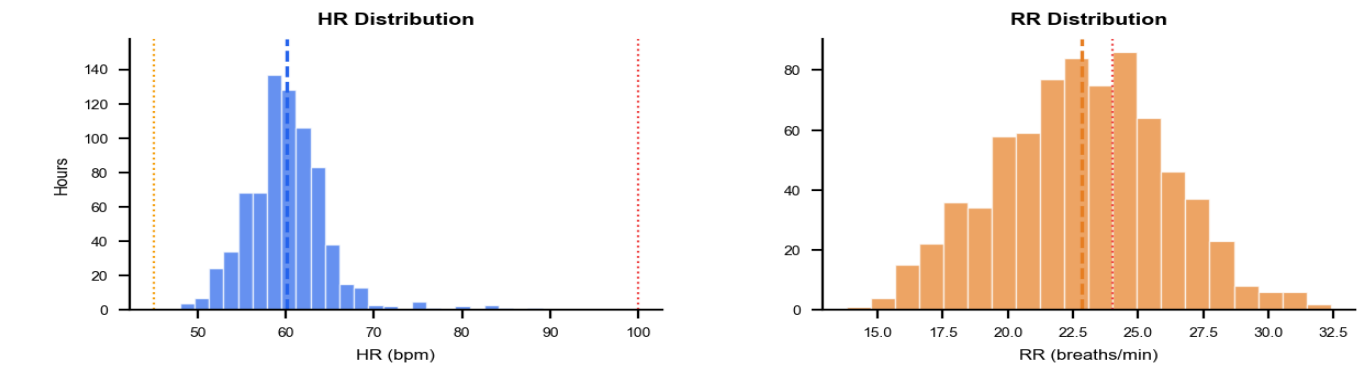
Clinical Pattern Observations:

- 1. Sustained finding: 14h continuous Elevated Breathing on May 05.
- 2. Breathing variability: RR spread 10 brpm (17 to 27).
- 3. Clustered pattern: Episodic events on 6 of 32 days (18%).

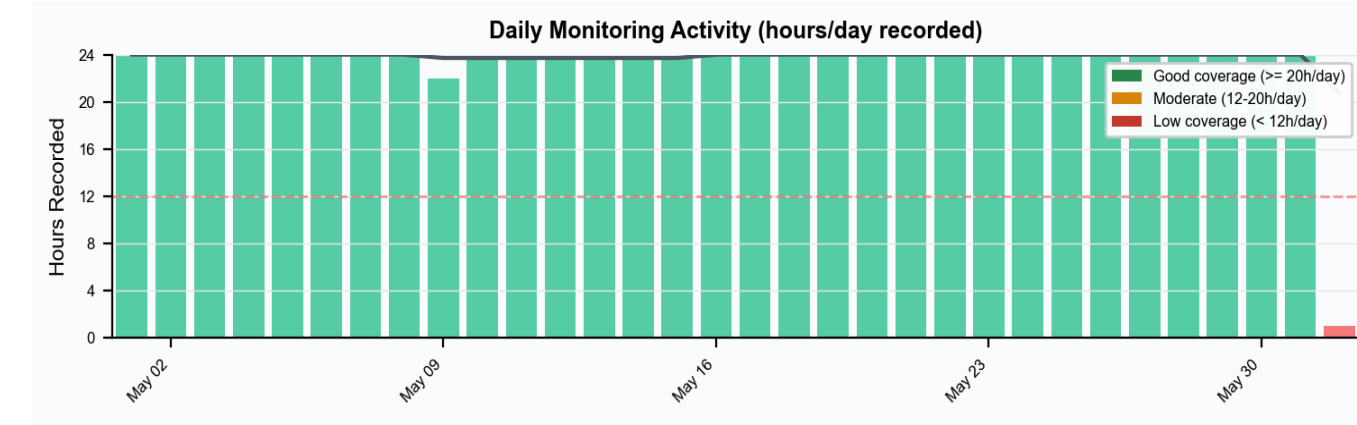


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode RR episode Recorded, no episode No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.